

NF2 Case Study — Skeleton & Build Guide

The scaffold the next 5 weeks fills in. Each section says what goes here, what evidence proves it, and which weekly deliverable feeds it. Write the case study into THIS structure as you go — don't synthesize from scratch at the end.

Locked scope (do not reopen)

- **One flow:** Unified Disease Timeline → Doctor-Ready 1-page Summary as its payoff screen.
 - **Five core screens:** Home · Log Event · Timeline · Generate Summary · Summary view.
 - **AI MRI parser:** mentioned as a *future ambition* in the reflection only. Not a designed screen. Reason: evaluators can't see UX credit in invisible AI, and you can't validate parsing without real data.
 - **Cut from this case study:** mental-state logging product, research library, sign-language module. They may appear as one line in "what I left out and why" — proof of disciplined scoping, which evaluators reward.
 - **Xuan's role:** clinical sanity-check and a named stakeholder quote. NOT co-owner of the survey. Her PhD QoL work is a separate instrument.
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Disclosure stance (applies to the whole write-up)

Your NF2 / deafness is the *credibility engine*, not the headline. Lead each section with the design problem and the user; let your lived experience surface as method ("I'm an NF2 patient, so I could pressure-test recruiting and phrasing in ways an outside designer couldn't"). Never open the case study with your diagnosis. Skill-first, lived-experience-as-proof.

0. Hook / TL;DR (*write LAST, top of page*)

One sentence on the problem, one on the solution, one hero image (the final Summary screen).

- *Who it's for · what breaks today · what you designed · the single outcome metric or quote.*
 - Feeds from: Week 5 final artifacts.
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1. Context & My Role

- The setting: rare disease, fragmented cross-border care, patient as lifetime record-keeper. **Plain language — no "Project Aegis," no ESG, no "Principal Investigator" branding.**
- Your role: end-to-end UX (research → synthesis → IA → wireframes → test). Team of 2.
- Constraints made honest: rare-disease = small sample; solo Figma build; 5-week timebox. State these — owning constraints reads as senior.
- Feeds from: this doc + plan dates.

2. The Problem (North Star)

- The *ultra-specific* reframed statement. Replace the abstract "principal investigator" line with a named-persona sentence, e.g.: "[Persona] needs to assemble scattered MRI reports, symptoms, and surgeries into one view because in a 10-minute consult she can't reconstruct her own timeline from memory."
- Initial assumptions, dated, written BEFORE interviews (intellectual honesty = you'll later show which were wrong).
- Feeds from: Week 1 (assumptions) → Week 3.4 (refined North Star).

3. Research

- **Method:** secondary research (3 challenges from papers), competitive audit (MyChart / PatientsLikeMe — what fails for rare/multi-tumor), survey, interviews. State numbers honestly; never claim significance on small n.
- **The survey as a thinking artifact:** include the "begin with the end / every question drives a design decision" story and the DeepSeek-vs-critical-mindset anecdote — that's a differentiator. (See [Case_study_story_tracker.md](#).)
- **Raw quotes:** 5–8 anonymous verbatim quotes. Hiring managers value these over polished charts.
- Feeds from: Week 1 (1.1–1.4), Week 2 (all).

4. Synthesis (Define)

- **1 primary persona** — real, from data, with goal + frustration + accessibility profile (from survey Q5).
- **User journey map** — one scenario: "the 2 days before a check-up," emotional highs/lows.
- **Top 3 insights → 5 "How Might We" statements.**
- (*Optional, for Xuan:*) a separate "operational friction" note for her PhD — kept OUT of the portfolio narrative so it doesn't dilute the design story.
- Feeds from: Week 3 (3.1–3.4), Week 4.1.

5. Design (Ideate & Build)

- **Information Architecture** — how a user gets from Home → Timeline → Summary. One diagram.
- **User flow** for the hero path: Log an event → see it on Timeline → Generate doctor summary.
- **Crazy Eights → low-fi wireframes** (5 screens) → hi-fi in Figma.
- **Accessibility as the design spine, tied to survey data:** ≥48dp touch targets (hand tremor), high-contrast / large type (vision), no parallax or fast motion (vestibular), captions/visual alerts (deafness). Every choice cites a survey number. This is your moat — make it explicit.
- Feeds from: Week 4 (4.1–4.4).

6. Validation

- Guerrilla usability test with 2–3 original interviewees: "point to where you'd log a headache."
- 3–5 iterations, shown as before/after.
- Feeds from: Week 5 (5.1–5.2).

7. Outcome & Reflection

- What the final flow does; the one moment that proves it (the 60-second handoff).

- **What I left out and why** (the cut features — disciplined scoping).
 - What I'd validate next; where the AI parser *could* go later.
 - Honest learnings, including what your assumptions got wrong.
 - *If reusing for PhD*: frame as evidence of research aptitude (structured inquiry, synthesis, accessibility scholarship) — NOT as a substitute for a paper.
 - Feeds from: Week 5 (5.3–5.4).
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Map: weekly plan → case-study section

Week	Plan deliverable	Fills section
1	Desk research, competitive audit, survey, assumptions	1, 2, 3
2	Survey responses + interviews	3
3	Affinity map, persona, journey, refined North Star	2, 4
4	HMW, IA, user flow, wireframes	4, 5
5	Usability test, iterate, write-up, polish	6, 7, 0

Open decisions to resolve before Week 2 goes live

1. Reconcile EN vs CN survey (residence options + the caregiver rationale currently contradict each other).
2. Re-frame survey Q10 as MVP *prioritization*, not feature discovery (it's leading as written).
3. PwC " " — go / no-go before committing any hours. (Separate session.)